



Policy

Suspected Spinal Injury – Investigation and Management (OPH)

This policy is under review and remains the current endorsed policy until the review is complete

Scope

Site	Service/Department/Unit	Disciplines
OPH	Hospital-Wide	All Staff

Introduction

Osborne Park Hospital (OPH) is not equipped to treat acute spinal injuries and as such, after clinical assessment by a medical officer indicates a suspected spinal injury, proceedings to transfer the patient to a tertiary site must commence at the earliest opportunity.

Strong consideration must be given to the likelihood of a concurrent head injury.

Imaging for acute spinal injury, including imaging for concurrent head injury, is not performed at OPH.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Procedure

Spinal injuries have the potential to cause long term disability with a marked decline in quality of life.

Risk of further injury to the spine can be reduced by providing stabilisation to the neck and spine, as well as limiting neck and spine range of motion.

Strong consideration must be given to the likelihood of a concurrent head injury.

Following an incident in which a spinal injury may have occurred, the first attending clinician will establish the likelihood by assessing mechanism of fall or incident, intensity, and location of pain.

Signs and symptoms of a spinal injury include:

- Pain at or below site of injury

- Loss of sensation, or abnormal sensation such as tingling in hands or feet
- Loss of movement or impaired movement below site of injury
- If the patient is unconscious following a head injury, always suspect a spinal injury.

If a spinal injury is likely, the patient must not be moved, unless in danger, until assessment has occurred.

Support head, neck, and spine in a neutral position always.

Maintaining an open airway is always a priority. If airway cannot be established or maintained in a neutral position, then a supported jaw thrust is recommended. The patient must be further assessed as soon as possible by a medical officer.

Maintain comfort of the patient and transfer via ambulance to SCGH for further assessment.

All precautions for the prevention of further spinal injury must remain in place until cleared by a medical officer.

Authorisation & Version Control

Policy Sponsor	Osborne Park Hospital Site Lead/Coordinator of Nursing				
Policy Contact	Osborne Park Hospital Site Lead/Coordinator of Nursing				
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Standards Applicable					
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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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